جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سميه علي محمد ابو موسي
المحافظة : الجيزه **الحي / المدينة :** كرداسه **العنوان :** =
رقم البطاقة : 26211032101983 **رقم التليفون :** 01152775650 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 03-11-1962 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سميه علي محمد ابو موسي
National ID: 26211032101983

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تحقيق الشخصية



سميه

على محمد ابوموسى

قرية كرداسة

مركز كرداسة - الجيزه

١٩٦٢/١/٣

٢٩٨٣٠١٢١٠٣١١٢٦٢

KU8209593



٢٩٨٣٠١٢١٠٣١١٢٦٢ ٢٠٢٤/١٠

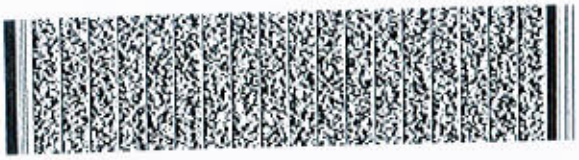
ربة منزل



أنثى مسلمة متزوجة

عبدالرحمن عبدالواحد محمد جبر

١ البطاقة سارية حتى ٢٠٣١/١٠/١٨



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient's Name:	**Samiha Ali Mohammed Abu Moussa**
Date:	**2026-07-12**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED :

A reported case of managed right breast cancer, no available clinical data or previous examination

I- CHEST:

- Bilateral basal atelectatic bands are noted.
- No pulmonary nodules, masses or cavitary lesions.
- Subcentimeteric mediastinal lymph node enlargement.
- No pleural or pericardial collection.
- Average cardiac size with atherosclerotic calcific foci of the aorta
- Small hiatus hernia
- Enlarged left thyroid lobe with calcified nodules are noted

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- The gall bladder is average distended with multiple radiodense stones, the largest is seen measures about 8.5 mm
- Normal non-contrast C.T appearance of the spleen, pancreas, right suprarenal glands. Bulky left suprarenal gland
- The urinary bladder shows no evidence of gross masses or diverticula or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Multiple subcentimeteric abdominal and pelvic lymphadenopathy.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Samiha Ali Mohammed Abu Moussa**
Date:	**2026-07-12**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- No ascites.
- **The examined skeleton** left acetabulum sclerotic lesion with multiple tiny lytic lesions **for further bone scan**

Opinion:

- **No pulmonary or hepatic deposits.**
- **Bilateral basal atelectatic bands.**
- **Calcular gall bladder.**
- **Bulky left suprarenal gland.**
- **Left acetabulum sclerotic lesion, multiple tiny lytic lesions for further bone scan**

Dr.Nourhan Ramzy

Dr.Nourhan Ramzy

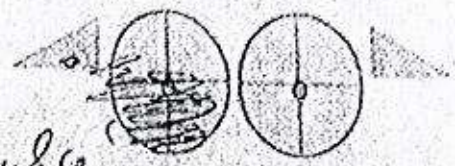
CT Scan
[[DOCTYPE:CT_SCAN]]

١٨ - ١٠ - ٢٠٢١

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

MDT FORM

Name	
Age	٥٢
MRN	٢٠٢١٠٢٤١٠١٩٨٢
ID	٢٨١١٥٠٧٧٠٦٥
Co morbidity	C.V.I / DM



Clinical Data

Bx: IDC & LI	SM:
IHC: ER: - ve PR: - ve Her1: - ve ++ Ki67:	Mets Workup: PET - Pos b/l irresectable
c T4b N1 M1 Stage IV	
COMMENT: 2019: Pt. Br C treated by AC x 5 then last Rt br op. → declined 2021: Restarted Taxol + herceptin SC + palliative Rt to b/l breast → good response however, still irresectable	

MDT OUTCOME & PLAN

Br - palliative 1st line Trastuzumab IV + Abemaciclib Tab

- No role of surgery

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

Pathology Report

Name: سوميه على ابو موسى **Age: 54 years.**

Clinical data: Left breast cancer

Specimen type: Slide revision.

Date (Received):23 /6/2021. **Date(reported): 24/6/2021**

Lab. Number:SR-264 / 21 **Hospital Number**

Gross description:

One block (سميه علىA74 Sept+2019) were received

Microscopic examination:

Sections examined revealed an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells with moderate cellular and nuclear atypia. Stromal desmoplasia is moderate.

Diagnosis:

Left breast cancer. Slide revision

Invasive duct carcinoma grade II

Marker study:

ER: No nuclear reaction. Negative, score (0/8)

PR: No nuclear reaction. Negative, score (0/8)

Her2: Strong, complete membrane reaction in more than 10% of tumour cells. Positive, score 3+

Ki67: 35%

Prof. Dr. Lubna Omar

Dr. Abdulrahman Ibrahim

قسم الباثولوجيا

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient's Name:	**SOMAIA ALI MOHAMED**
Date:	**2026-03-28**
Referred by:	Prof.Dr.
Hospital:	**CT CAP Goustave Roussey International Egypt**

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings: Reported case of breast cancer compared to last study dated on 31/05/2025

I Chest: -

- **Still noted small nodule 4mm at posterior segment of right upper lobe likely non specific**
- **No sizable pulmonary nodules .**
- **Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.**
- **No hilar or mediastinal lymph nodes.**
- **No evidence of pleural effusion, pleural thickening or masses seen.**
- **Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.**
- **Normal CT appearance of the thoracic aorta and the great vessels.**

II Abdomen & pelvis:

- **The liver is of average size with homogenous low CT attenuation . No definite focal lesions or contour irregularities.**

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient's Name: SOMAIA ALI MOHAMED

Date: 2026-03-28

Referred by: Prof.Dr.

Hospital: CT CAP Goustave Roussey International Egypt

- Calcular gall bladder .
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen…
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Under filling of the urinary bladder with thickened wall , hindering examination.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

No definite worrisome lytic focal lesions.

No vertebral structural collapse .

Opinion:

Known case of breast cancer shows:

Overall stationary course as compared to last study dated on 31/05/2025

DR/Reham Khedr,,,MD

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

A. . Khalil
Pr sse. f Pathology
F ulty of Medicine
Kasr El-Aini

دكتور
عبدالله محمود خليل
أستاذ الباثولوجي – كلية طب قصر العيني
مستشار الباثولوجيا بمعهد السرطان القرمسي
عضو أكاديمية الباثولوجيا الدولية

Patient Name: سميه علي ابو موسى

Referred by Prof. Dr.: I. M. EL GHAZALANY

Nature of the Specimen: INCISIONAL BIOPSY.

Clinical Diagnosis: BREAST MASS

Receiving Date: Sept. / 2018 **Code Number:** A 74

PATHOLOGY REPORT

GROSS PATHOLOGY:
Pieces of pale yellow focally hemorrhagic tissue 3.5x3 cm collectively, one piece showed attached skin ellipse 0.7x0.3 cm. Totally submitted.

HISTOLOGY:
Sections showed pieces of breast tissue invcived by tumorous growth focally covered by skin ellipse. The tumorous growth was formed of moderately pleomorphic and cohesive neoplastic ductal cells having large vesicular nuclei with hyperchromatic chromatin pattern and prominent nucleoli. The cytoplasm was adequate and oesinophilic. They were displayed in sheets, clusters, cords and distorted glandular pattern. The stromal background was fibrotic and infiltrated by lymphocytes.

DIAGNOSIS:
INVASIVE DUCT CARCINOMA (G II-III) WITH MODERATE LYMPHOCYTIC REACTION KEEPING WITH ATYPICAL MEDULLARY TYPE.

P.O.:
1- Detection of ER, PrR, Her-2 and Ki-67 in paraffin tissue sections is now available on request.
2- Slide enclosed.

A.M. Khalil, MD

عمارة الأعظمية: ٧٠ شارع المنيل - ميدان الباشا – المنيل - القاهرة تليفون:٢٣٦٢٠١٠٧-٢٣٦٣٠٧١١-٢٥٣٢٠٤٣٥فاكس:٢٥٣٢٠٣٦٩
El-Azamia Building:70 El-Manial St. El-Pasha SQ., El-Manial, Cairo Tel: (02)23620107-(02)23630711-(02)25320435-02)25320369

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name:	***Somaiah Ali Abou Mousa***	***Patient ID:***	***447129***
Prof. Dr:	***Amr Yehia Sakr***	***Exam Date:***	***3/6/2021***
Accession:	***210601283***		

Whole Body PET/CT Examination

History and Clinical Data:-

Patient data:-

Female patient 56 years old, Weight: 76 Kg, Height: 157 cm, Blood Glucose level: 90 mg/dl

Radiopharmaceutical:-

F-18-FDG 7 mCi injected I.V.

Clinical background:-

- History of right breast cancer following chemotherapy
- Last chemo-therapy cycle ended 2019.

Purpose of the study:-

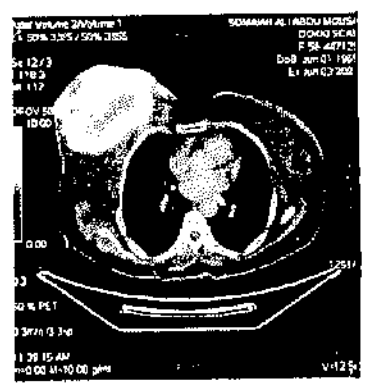
- Detection of any viable tumors.

Technique:-

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~ 1.9.
- Mediastinal blood pool average SUV ~ 1.4.

Findings:-

I) ***Breasts and axillae:-***



- A large metabolically active (FDG avid) multiple right breast necrotic masses and nodules nearly occupying the whole breast parenchyma invading the skin and nipple with overlying skin ulceration, they measures up to 12.4 x 12.6 x 14 cm with max SUV ~ 22.43.
- Low grade metabolically active left breast skin thickening with no underlying metabolically active mass lesions.
- Metabolically active (FDG avid) bilateral axillary and retro pectoral lymph nodes are seen measuring up to 3 cm with max SUV ~ 14.32 (right axillary lymph nodes) while the left axillary lymph nodes are the most active showing max SUV ~ 20.83 and measures up to 2.6 cm.

II) ***Head:-***

- Scanned brain sections show uniform bio-tracer distribution in the cerebral and cerebellar hemispheres.

PET Scan
[[DOCTYPE:PET_SCAN]]

Patient Name:	*Somaiah Ali Abou Mousa*	*Patient ID:*	*447129*
Prof. Dr:	*Amr Yehia Sakr*	*Exam Date:*	*3/6/2021*
Accession:	*210601283*		

III) ***Neck:-***
- Low grade metabolically active right lower deep cervical / supraclavicular lymph nodes are seen measuring up to 1.6 cm with max SUV ~ 5.13.
- No metabolically active (FDG avid) masses are noted.

IV) ***Chest:-***
- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- Right basal atelectatic bands.
- No metabolically active (FDG avid) pleural effusion is noted.
- Low grade metabolically active subcentimetric prevascular lymph node is seen showing max SUV ~ 2.74.
- Mild cardiomegaly. Echocardiography is advisable.

V) ***Abdomen and pelvis:-***
- Average sized liver showing no metabolically active (FDG avid) focal lesions.
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active (FDG avid) porta hepatis, mesenteric, retroperitoneal or pelvic lymph-nodes are noted.
- Gall bladder stones are noted.
- Right renal simple cortical cyst.
- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.
- No ascites.

VI) ***Axial and scanned Appendicular Skeleton***
- No (FDG avid) metabolically active osteolytic or osteosclerotic lesions are noted.

Conclusion:
- ***Positive study for metabolically active locally invasive right breast mass (breast cancer) with metabolically active supradiaphragmatic nodal metastatic deposits.***

Thank You for Your reference
PET/CT Unit

Dr. Amany Fathy, MSc
Revised by Dr / PM
Typed by , Eiss

Radiology consultant
Prof. Dr. Dalia Khaled, MD

PET Scan
[[DOCTYPE:PET_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Somaya Ali Mohamed Abomosi	Date : 16/07/2022
Exam : MAMMOGRAPHY BOTH	Age : 59 Year(s)
Medical ID: 2194/2021	Pat. ID: 15087

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right malignant mass, with no available data given by her clinician neither by the patient, and no previous study done (according to patient words)

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Right marked discripency in size with diffuse skin thickening, with marked lower quadrants distortion noted. US showed lost nipple-areolar complex with parenchymal distortion seen 4-8:00
- Left marked heterogenous parenchyma with no suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no left solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex.
- Left non-specific axillary LNs.
- Right axilla marked distortion seen, where axillary lymph nodes cannot be properly examined right enlarged supraclavicular lymph noce measuring 8.5x4.9 mm

Opinion:

- Right marked distortion with ipsilateral pathological looking supraclavicular lymph node. For clinical correlation

Yours truly

Signature Dr. Yasmin Nada

16/07/2022 12:04 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Somaya Ali Mohamed Abomosi	Date : 15/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 2194/2021	Pat. ID: 15087

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Status post right mastectomy.
Compared to the last study dated 31-5-2025 ,the current study revealed:
(I) Chest:

- **Complete resolution** of the right upper lung lobe posterior segment nodule.
- Bilateral lower lobar basal fine reticulations.
- Patent central air ways with well inflated both lungs.
- No parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:
Stationary course of the previously noted:

- The liver is average in size showing homogenous CT density with no focal lesions.
- Calcular gall bladder with multiple gall stones.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

OPINION:

- **Follow up status showing the above detailed changes.**

Dr.Sara Mahmoud, MD

Dr.Sara Ali

Dr.Sara Ali

CT Scan
[[DOCTYPE:CT_SCAN]]